

ESSENTIAL NEWBORN CARE



**“ ZERO SEPARATION” AND
“ DO NO HARM”**



ZERO SEPARATION



DO NO HARM



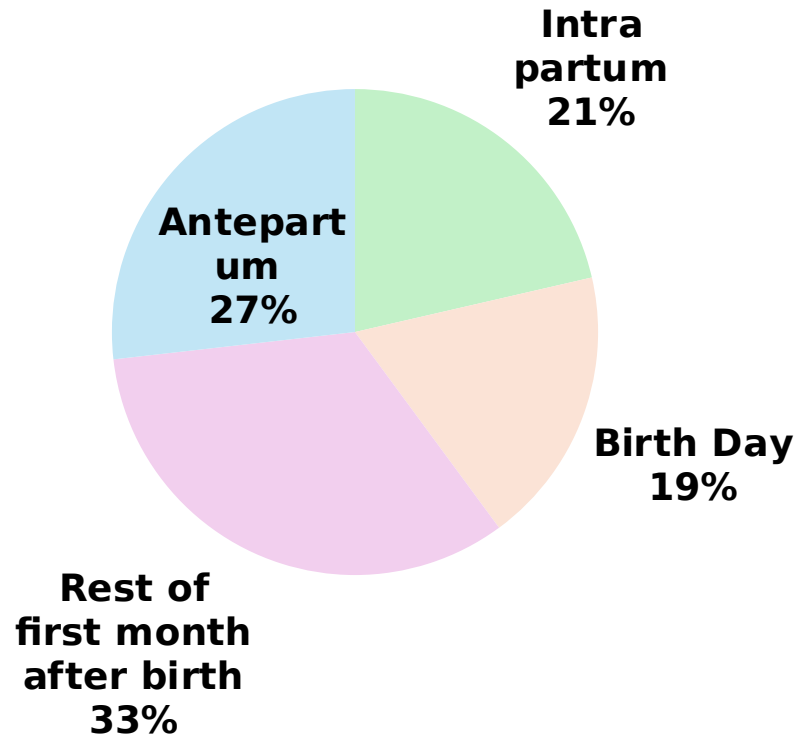
BETTER START
BETTER FUTURE

Topic: 1



Importance of Care for Newborn at Birth

Neonatal mortality and stillbirths (5.5 million)



- Most of the neonatal deaths occur during delivery or on the day of birth
- Even for the deaths occurring in the rest of neonatal period, care given at the time of birth is an important influencer
- A lot of deaths of newborns due to hypothermia, asphyxia and infection can be prevented by providing essential newborn care to all newborns immediately after birth



ESSENTIAL NEW BORN CARE (ENBC)



Every Step Counts for a Healthy Start

1

Call out the time of birth.



2

Deliver the baby onto a warm, clean and dry towel or cloth and keep on mother's chest and abdomen (between the breasts).



3

Assess the baby's breathing while drying.*



4

Immediately dry the baby with a warm clean towel or piece of cloth.



5

Clamp and cut the umbilical cord when cord pulsation stops or in 1-3 minutes (delayed cord clamping).



6

Leave the baby between the mother's breasts to start skin-to-skin care.



7

Place an identity label on the baby.



8

Cover the baby's head with a cap. Cover the mother and baby with a warm cloth.



9

Encourage the initiation of breastfeeding.



10

Administer Inj. Vitamin K intramuscular to baby according to weight.



* - If the baby is not crying or breathing well, resuscitation needs to be carried out.



BENEFITS OF ESSENTIAL NEWBORN CARE

ENBC

BENEFITS

ESSENTIAL ACTIVITIES



Warmth



Infection prevention



Mother's milk



Breathing



Prevent neonatal anemia

1



Place on dry and warm towel

2



Immediate drying

3



Cover baby's head with cap

4



Skin to skin contact

5



Clean cord cut, cord tie and dry cord stump

6



Early initiation of breast feeding

7



Assess while drying

8



Delayed cord clamping



NEWBORN CARE

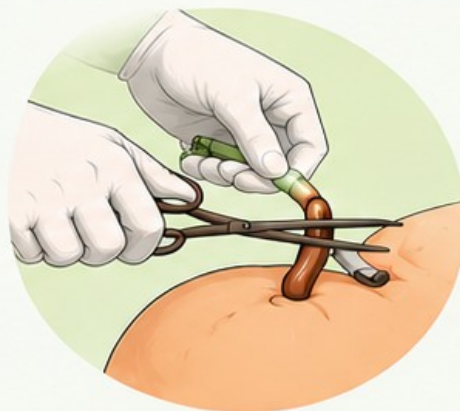


Essential care for every newborn, every time



CLEAN CHAIN

- 1 Clean Hands
- 2 Clean mother
- 3 Clean delivery surface
- 4 Clean cord cutting
- 5 Clean cord clamp/Cord thread to tie
- 6 Clean and dry cord



BREAST FEEDING

Immediately after birth within 30 mins

- 1 Uterine contraction
- 2 Emotional bonding
- 3 Boost baby immunity
- 4 Reduce PPH
- 5 Reduce risk of sudden infant death syndrome



WARM CHAIN

- 1 Warm Delivery room (26-28 degree C)
- 2 Warm Resuscitation
- 3 Immediate drying
- 4 Skin to skin contact
- 5 Postponed bathing for 24 hours.
- 6 Breast feeding
- 7 Appropriate bedding
- 8 Keep baby and mother together
- 9 Warm transportation
- 10 Awareness-raising of healthcare provider



Note: Use **SAMMPurNA** kit for maintaining warm chain.



INJ. VITAMIN K

All new-borns delivered in the last 24 hours either in the facility or at home should receive Inj.vit K.

Drug:

Inj.Vit K1 (Phytonadione): 1mg /ml

Dosage:

Less than 1kg baby- 0.5 mg

More than 1 kg- 1 mg



1 ml syringe
with 26G
needle



A warm start, a healthy future
Care today. health forever





INJECTION VITAMIN K FOR NEWBORN

A SIMPLE STEP FOR A SAFE START



WHO WILL RECEIVE?

All newborns delivered in the facilities at all levels (both public and private)



PREPARATION TO BE USED

Injection Vitamin K1 (Phytonadione):
a) 1 mg/1 ml
b) 1 mg/0.5 ml



DOSE TO BE GIVEN

- Birth weight 1000 gm or more: 1 mg
- Birth weight less than 1000 gm: 0.5 mg

BENEFITS OF VIT K



- Prevents hemorrhagic disease of newborn



SITE AND ROUTE OF INJECTION

- Antero-lateral aspect of the thigh, intramuscular injection



WHO WILL GIVE?

- Medical Officer, staff nurse or ANM



WHERE IT WILL BE GIVEN

- In labour room
- It can be given in post-natal ward if missed in labour room
- In case of referral the injection should be given at the SNCU/NBSU



WHEN IT WILL BE GIVEN

- Soon after delivery, ensuring skin-to-skin contact with mother and initiation of breast feeding
- Not later than 24 hours of birth



LOGISTICS REQUIRED

- 26 gauze needle and 1 ml syringe



STORAGE

- Room temperature in a dry place



RECORDING

- Labour room register
- Case sheet
- Referral slips
- Discharge ticket of the newborn



A SMALL INJECTION TODAY, A LIFETIME OF PROTECTION TOMORROW.





ZERO SEPARATION



• Keep Mother and Baby Together, Right from the Start •

ZERO SEPARATION



Prevents Hypothermia



Increases breast milk secretion



Reduces newborn infection



Provides environment for brain development



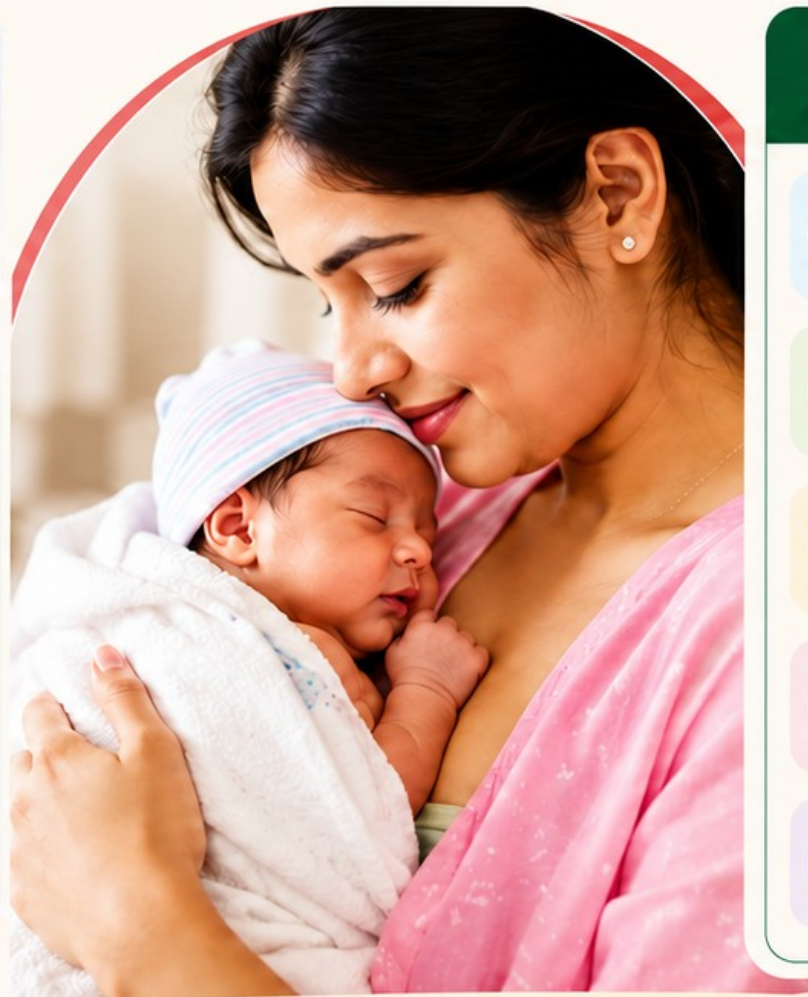
Decreases stress of Baby and mother



Provides multimodal stimulation



Improves mother child bonding



PRACTICE OF SIMPLE ACTIVITIES CAN IMPROVE NEWBORN SURVIVAL



Delayed Cord Clamping



Early Skin to skin care
(start at LR / OT)



Early Initiation of Breast Feeding



Exclusive Breast Feeding



Hygiene and sanitation



Simple Actions Today, Stronger Tomorrow





Facilities available at present for care of Newborns



FACILITY	SERVICE PROVISIONS
 NBCC at all delivery points including each at LR and OT	• Care for all newborns • Routine care & Resuscitation
 NBSU at selective FRUs	• Stabilisation of all sick newborns including inborn and Outborn • Care of jaundiced, small and less sick babies
 Newborn Screening at all DPs	• Equipments for baby like Digital weighing scale, Infantometer, HC Tape, Birth Defect register etc.
 SNCU	• Care of all sick babies requiring IV Fluid, Oxygen, OG Feeding and monitoring

IMPORTANCE OF ASEPSIS



Prevention of infection is more **cost effective** than treating infection in **neonates**.

BASIC REQUIREMENTS FOR ASEPSIS IN A BABY CARE AREA



Running water supply



Soap



Elbow or foot operated taps



Strict hand washing



Avoid overcrowding, optimal number of health providers for care of more babies



Plenty of disposals



Strict adherence to good housekeeping and asepsis routines



Advice mother to cut her nails



HAND WASHING: CLEAN HANDS, HEALTHY LIFE

— ♥ — A Small Step for You, A Big Protection for All — ♥ —



STEPS IN HAND WASHING

1



1. Rub palms

Rub hands palm to palm.

2



2. Back of hands

Rub back of each hand with palm of other hand with fingers crossed.

3



3. Back of fingers

Rub with back of fingers to opposing palms with fingers linked.

4



4. Thumbs

Rub each thumb clasped in opposite hand using a twisting movement.

5



5. Fingertips

Rub tips of fingers in opposite palm in a circular motion.

6



6. Wrists

Rub each wrist with opposite hand thoroughly.

WHY HAND WASHING IS IMPORTANT?



Prevents infections and diseases



Stops the spread of germs



Protects you, your family and community



Keeps hands clean and healthy

NOTE

Rinsing health with alcohol is
NOT A SUBSTITUTE
for Hand Washing.



ADDITIONAL REMINDERS

- Wash hands before eating or handling food.
- Wash hands after using the toilet.
- Wash hands after coughing, sneezing or blowing nose.
- Wash hands after touching animals or their waste.



WASH HANDS WITH SOAP AND CLEAN WATER
DURATION: AT LEAST 20 SECONDS



Clean Hands Today, Healthy Tomorrow!

OTHER MEASURES TO PREVENT INFECTION IN NEWBORN



Avoid overcrowding



• Sick persons should not be engaged in baby care



• Don't wipe the mouth till 6 month



• Feed with only breast milk



• Keep baby with mother for maximum time



• Do not keep long or polished nails.



• Remove threads, more bangles and rings

Monitoring of Baby (Delivery Case-sheet)

Notes for Mother:

Any Conditions Present :

Notes for Baby:

Activity :

Respiratory Rate :

Colour :

Tone :

Others (specify).....

Assessment of Postpartum Condition

Mother	15 min	15 min	15 min	15 min	30 min	30 min	6 hrs	6 hrs	6 hrs
BP (mmHg)									
Temp(°C/°F)									
Pulse (per min)									
Bleeding PV Normal-N Excessive-E	N ☐ E ☐	N ☐ E ☐	N ☐ E ☐	N ☐ E ☐	N ☐ E ☐	N ☐ E ☐	N ☐ E ☐	N ☐ E ☐	N ☐ E ☐
Uterine Tone Soft-S Contracted-C	S ☐ C ☐	S ☐ C ☐	S ☐ C ☐	S ☐ C ☐	S ☐ C ☐	S ☐ C ☐	S ☐ C ☐	S ☐ C ☐	S ☐ C ☐
Urination volume per hour for minimum 24 hours									
Baby									
Resp rate (per min)	N - 40 - 60 / min								
Temp (°C/°F)	N - 36.5°C - 37.5°C								
Features of Hypoglycemia									
Chest Movement									
Any other danger sign									
Breastfeeding									
Suckling (Yes/No)									
Cord condition	Normal ☐ Bleeding ☐ Other ☐								
Congenital anomalies									
Immunization									

DELIVERY NOTES

Delivery date: / / Time:

Delivery conducted by: Designation:

Type of delivery: Normal ☐ Assisted: Ventouse ☐ LSCS ☐ Others (specify) Early fetal death ☐ Single ☐ Still birth: Fresh ☐ Macerated ☐ Neonatal death ☐ Twin/ Multiple ☐ Outcome: Live birth ☐ Early fetal death ☐ Single ☐ Still birth: Fresh ☐ Macerated ☐ Neonatal death ☐ Twin/ Multiple ☐ Weight of Still birth (in gram) Baby delivered on mother's abdomen ☐ Yes ☐ No ☐ Episiotomy: Yes ☐ No ☐ AMTSL performed: Yes ☐ No ☐ Uterotonic used: Inj. Oxytocin ☐ Tab Misoprostol ☐ Others (specify) Complications, if any: PIH/PET/ Eclampsia ☐ Prolonged labor ☐ Obstructed labor ☐ PPH ☐ Sepsis ☐ Fetal distress ☐ Perineal injury Type I, II, III, IV ☐ Others (specify)

BABY NOTES

Sex of the baby: Male ☐ Female ☐ Birth weight: (in gram) Pre Term ☐ Term ☐ IUGR ☐ Did the baby cry immediately after birth : Yes ☐ No ☐ Delayed Cord Clamping if Baby Cried after Birth ☐ Birth Asphyxia : Yes ☐ No ☐ Did the baby require resuscitation with Bag & Mask: Yes ☐ No ☐ Was the baby dried immediately after birth : Yes ☐ No ☐ Early Initiation of Breast feeding: Yes ☐ No ☐ If yes, steps performed Any congenital anomaly: Refd. to SNCU Yes ☐ No ☐ Injection Vitamin K1 administered: Yes ☐ No ☐ Vaccination done: BCG ☐ OPV ☐ Hep B ☐

Foot-print of Baby

ନବଜାତ ଶିଶୁର ଦୈନିକ ସାଙ୍ଗ୍ୟ ବିବରଣୀ

NEW BORN MONITORING RECORD

Name of Mother : (ମା'ର ନାମ) Date of Birth : (ଜନ୍ମତାରିଖ)

Address : (ଠିକଣା) Time of Birth : (ଜନ୍ମସମୟ)

Sex of the Baby (ପୁଅ / ଝିଅ) Gestation-Preterm/Term/Post-Term

Any cong anomaly : Birth weight (in grams) N / LBW / VLBW / ELBW

DATE					
AGE (HOURS)					
MORNING / EVENING - TIME					
1. PROPER BREAST FEEDING (Y/N)					
2. WEIGHT					
3. ACTIVITY (ACT/ LETH)					
4. PERIPHERY (Cold Warm)					
5. COLOUR (PINK / BLUE)					
6. PULSE RATE					
7. RESP RATE					
8. SEVERE CHEST RETRACTION (Y/N)					
9. GRUNT (Y/N)					
10. TEMPERATURE					
11. JAUNDICE					
12. URINE (Time of 1st unination & Frequency)					
13. STOOL					
14. IMMUNISATION BIRTH DOSE (Y/N)					
15. OTHERS (SPECIFY)					
16. WEARING OF DRESS / CAP SOCKS					
17. MOTHER KNOWS HAND HYGIENE & KMC					

NB : C = COLD, W = WARM, P = PINK, PC = PERIPHERAL CYANOSIS, CC = CENTRAL CYANOSIS; PLR : PALLOR

1. KEEP BABY WITH MOTHER (ZERO SEPARATION)
GOLDEN RULE : 2. WASH HANDS BEFORE AND AFTER HANDLING A BABY.
3. EXCLUSIVE BREAST - FEEDING UPTO SIX MONTHS OF AGE.



NEWBORN SCREENING

for early identification
and management of

BIRTH DEFECT

AT ALL DELIVERY POINTS



SCREEN

Visual assessment
of the newborn



IDENTIFY

Early identification
of birth defects



MANAGE

Timely management
and referral



**EARLY DETECTION.
BETTER OUTCOMES.**

DEFECTS AT BIRTH UNDER RBSK



Neural Tube Defect

Defects in the brain, spine or spinal cord.



Down Syndrome

A genetic condition caused by the presence of an extra copy of chromosome 21.



Cleft Lip & Palate / Cleft Palate alone

Opening or split in the upper lip, the roof of the mouth or both.



Talipes (club foot)

One or both feet turned inward or downward.



Developmental Dysplasia of the Hip

Abnormal development of the hip joint.



Congenital Cataract

Clouding of the eye lens present at birth.



Congenital Deafness

Partial or complete hearing loss present at birth.



Congenital Heart Diseases

Structural problems in the heart present at birth.

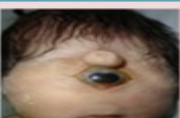
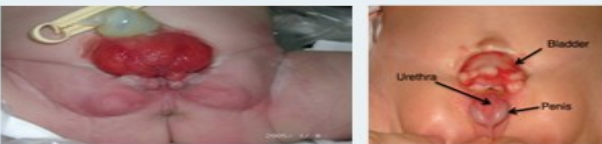


Retinopathy of Prematurity

Abnormal blood vessel growth in the retina in premature babies.



Apart from these, Congenital Hypothyroidism, Sickle Cell Anaemia, Beta Thalassemia have been kept as optional conditions depending upon their high prevalence in any of the States/UTs and availability of testing and specialized support facilities.

Nervous system	 Anencephaly Q00	 Spina Bifida with Meningomyelocele Q05	 Encephalocele Q01	 Hydrocephalus Q03	 Arhinencephaly / Holoprosencephaly Q041, Q042
Eye related disorders	 Anophthalmos Q110, Q111	 Microphthalmos Q112	 Congenital cataract Q120	 Congenital glaucoma Q150	Chromosomal disorders  Down syndrome Q90
Ear, face and neck	 Microtia II Q17.22	 Microtia III Q17.23	 Microtia IV / Anotia Q16.0	Oro- facial disorders  	
Genital disorders	 Hypospadias Q54			 Indeterminate sex/ Ambiguous genitalia Q56.4	
Urinary tract disorders	 Bladder exstrophy and / or epispadias			If a male child check the urinary stream  Posterior urethral valve	
Limb related disorders	 Limb reduction defects Q71-Q73			 Polydactyly Q69	
				 Syndactyly Q70	
				 Club foot – talipes Equinovarus Q660	

RoP IEC



LV Prasad Eye Institute

ଆପଣ ଜାଣନ୍ତି କି ?

ଯଦି ଆପଣଙ୍କ ଶିଶୁର ଜନ୍ମ ଓଜନ ୨ କେଜି ବା ତାହାରୁ କମ୍ କିମ୍ବା ସେ ମାଆ ପେଟରୁ ୩୪ ସପ୍ତାହ କିମ୍ବା ତା ଆଗରୁ ଜନ୍ମ ନେଇଥାଏ

ତେବେ ଶିଶୁଟି (ଆର.ଓ.ପି) ନାମକ ଏକ ଚକ୍ଷୁ ରୋଗ ଓ ଅନ୍ଧତ୍ୱ ର ଶିକାର ହୋଇପାରେ ।



ଉପାୟ: (ଆର.ଓ.ପି / ROP) ଚକ୍ଷୁ ବିଶେଷଜ୍ଞଙ୍କ ଦ୍ୱାରା ଚକ୍ଷୁ ପରୀକ୍ଷା



କେବେ? ଜନ୍ମର ୨ ରୁ ୪ ସପ୍ତାହ ମଧ୍ୟରେ
ମନେରଖନ୍ତୁ, ଭୁଲନ୍ତୁ ନାହିଁ (ଆର.ଓ.ପି / ROP) ପାଇଁ ୨ ୩୪



ଅଧିକ ସୂଚନା ପାଇଁ ଯୋଗାଯୋଗ କରନ୍ତୁ

୦୭୭୪-୨୬୫୩୦୦୧

୬୩୭୧୦୦୮୨୪୮, ୬୩୭୦୩୬୭୮୩୦



Take Home message



Delayed Cord Clamping



Early Skin to Skin Care (KMC)



Early Initiation of Breast Feeding



Exclusive Breast Feeding



Hygiene and sanitation



Early identification of Danger Signs and Referral



**Small interventions can give Big results,
Save the tiny baby who may help you in old age.**

